[redacted]

FAX

Date: [redacted]

TO: Recup

ATTN: _____

FAX #: _____

FROM [redacted]

DEPARTMENT: Social Services

PHONE # [redacted]

E-MAIL: _____

FAX # [redacted]

PAGES TO FOLLOW: _____

COMMENTS: Recup placement.
pending update for OT / PT notes.

Important Warning: This message is intended for the use of the person or entity to which it is addressed and [redacted] contain information that is privileged and confidential, the disclosure of which is governed by applicable law. If you are not the intended recipient, or the employee or agent responsible to deliver is to the intended recipient, you are hereby notified that any disclosure, copying or distribution of this information is Strictly Prohibited.

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[redacted]

[redacted]

[redacted]

ADMISSION/REGISTRATION

[redacted]

Preferred Name

Admit/Serv Dt Time	IP Admit Dt/Time	ED Arr Dt Time	Disch Dt Time	Room/Bed	Location	Service
[redacted]				[redacted]		General Medicine

Pt Class	Arrival Mode	Source	Priority	Primary Care Physician	Office Phone
Inpatient	ALS Ambulanc	Trauma	Emergency	None Pcp, MD	None

Chief Complaint	Admission Diagnosis	User
	Tibia/fibula fracture [S*	[redacted]

Emergency Physician	Office Phone	Attending Physician	Office Phone	Admitting Physician	Office Phone
[redacted]					

PATIENT

SSN [redacted]

			Religion	VIP	Conf
55 y.o.	F	Unknown	Unknown		

Race	Ethnicity	Preferred Language	Maiden Other Name
Other	Not Hispanic	English	[redacted]

Address: [redacted]

Alternative Address:
Care of:

Home Phone:

Cell Phone [redacted]

PATIENT EMPLOYER

Employer:
Address:

Work Phone:
Occupation:

GUARANTOR

Name: [redacted]
Address: [redacted]

Home Phone:
Relationship to Patient: Self

GUARANTOR EMPLOYER

[redacted]
Address:

Work Phone:
Occupation:

EMERGENCY CONTACT 1

Relationship to Patient: Daughter

Name: [redacted]
Address:
Home Ph:
Work Ph:
Cell Ph: [redacted]

EMERGENCY CONTACT 2

Relationship to Patient: [redacted]

[redacted]
Address:
Home Ph:
Work Ph:
Mobile ph: [redacted]

Insurance # 1

Payor/Plan: MISC AUTO INSURANCE/MISC AUTO INSURANCE

Subscriber [redacted]
Pat Rel to Subscriber: Self
Group Name: [redacted]
Group Numbe
Subscriber ID: [redacted]

Authorization

Auth:

[redacted]

[redacted]

Insurance # 2		Authorization
Payor/Plan [redacted] DOB: [redacted]	Subscribe [redacted] Group Nan Group Nur Subscribe	Auth:

OTHER INFO

Organ Donor: N Primary Isolation: No active Isolations	Influenza Vaccine this Season? Pneumococcal Vaccine ever?	Accident Occurrence: ACCIDENT/NO MEDICAL OR L[redacted]

[redacted]

[redacted]

[redacted]

[redacted]

Only active medications are shown.

Scheduled Meds Sorted by Name

fo[redacted]

Legend:

Given	Hold	Not Given	Due	Canceled Entry	Other Actions
Time	Time	(Time)	Time	~~Time~~	**Time-Action**

Inactive: X | Active | Linked: GO

Medications

[redacted]

acetaminophen (TYLENOL) tablet 1,000 mg Dose: 1,000 mg Freq: 3 times daily Route: PO [redacted] Admin Instructions: The max daily dose for patients less than 50 kg is 75 mg/kg/24 hours and for patients 50 kg or greater is 4000 mg/24 hours. Consider all source of Acetaminophen.	1036-Given	0900	[redacted]	[redacted]	0900	[redacted]	0900
	1500	1500	1500	1500	1500	1500	1500
	2100	2100	2100	2100	2100	2100	2100
bacitracin ointment Freq: 3 times daily Route: Topical [redacted] Admin Instructions: Apply to flank and pelvic abrasions TID, apply to occipital scalp abrasion BID.	1033-Given	0900	0900	0900	0900	0900	0900
	1500	1500	1500	1500	1500	1500	1500
	2100	2100	2100	2100	2100	2100	2100
enoxaparin (LOVENOX) syringe 30 mg Dose: 30 mg Freq: Every 12 hours Route: SC [redacted] Admin Instructions: Administer by deep subcutaneous injection. Inject into the left or right anterolateral or posterolateral abdominal wall. * * * BLACK BOX WARNING * * * SPINAL/EPIDURAL HEMATOMAS [redacted] RESULT AFTER NEURAXIAL ANESTHESIA (EPIDURAL/SPINAL ANESTHESIA) OR SPINAL PUNCTURE I Order specific questions: Indication DVT Prophylaxis	1033-Given	0900	0900	0900	0900	0900	0900
	2100	2100	2100	2100	2100	2100	2100
levETIRAcetam (KEPPRA) 100 MG/ML solution 500 mg Dose: 500 mg Freq: Every 12 hours scheduled Route: PO [redacted]	1033-Given	0900	0900	0900	0900	0900	0900
	2100	2100	2100	2100	2100	2100	2100
melatonin tablet 3 mg Dose: 3 mg Freq: At bedtime Route: PO [redacted]	2100	2100	2100	2100	2100	2100	2100

[redacted]

[redacted]

methocarbamol (ROBAXIN) tablet 500 mg Dose: 500 mg Freq: 3 times daily Route: PO [redacted]	1036-Given 1500 [redacted]	0900 1500 2100	0900 1500 2100	0900 1500 2100	0900 1500 2100	0900 1500 2100	0900 1500 2100
multivitamin tablet 1 tablet Dose: 1 tablet Freq: Daily Route: PO [redacted]	1034-Given	0900	0900	0900	0900	0900	0900
QUEtiapine (SEROquel) tablet 100 mg Dose: 100 mg Freq: 2 times daily Route: PO [redacted] Admin Instructions: * * * BLACK BOX WARNING * * * NOT FDA APPROVED FOR DEMENTIA-RELATED BEHAVIORAL DISORDERS; INCREASED MORTALITY IN ELDERLY PATIENTS WITH DEMENTIA - RELATED PSYCHOSIS (RISK INCREASED WITH DEHYDRATION). INCREASED RISK OF SUICIDAL THINKING AND BEHAVIOR IN CHILDREN, ADOLESCENTS, AND YOUNG ADULTS WITH MAJOR DEPRESSIVE DISORDER OR OTHER PSYCHIATRIC DISORDERS ESPECIALLY DURING 1ST MONTH OF TREATMENT.	1033-Given 2100	0900 2100	0900 2100	0900 2100	0900 2100	0900 2100	0900 2100
senna-docusate (PERICOLACE) [redacted] MG 1 tablet Dose: 1 tablet Freq: 2 times daily Route: PO [redacted]	1033-Given 2100	0900 2100	0900 2100	0900 2100	0900 2100	0900 2100	0900 2100
sodium chloride tablet 3 g Dose: 3 g Freq: 3 times daily Route: PO [redacted]	1033-Given 1500 2100	0900 1500 2100	0900 1500 2100	0900 1500 2100	0900 1500 2100	0900 1500 2100	0900 1500 2100
urea powder 15 g Dose: 15 g Freq: 3 times daily Route: PO [redacted] Admin Instructions: Mix with any fluid that patient will tolerate	1033-Given 1500 2100	0900 1500 2100	0900 1500 2100	0900 1500 2100	0900 1500 2100	0900 1500 2100	0900 1500 2100

Medications [redacted]

Continuous Meds Sorted by Name

fo [redacted]

Legend:

Given	Hold	Not Given	Due	Canceled Entry	Other Actions
Time	Time	(Time)	Time	~~Time~~	**Time-Action**

[redacted]

[redacted]

Inactive	Active	Linked
X		

Medications [redacted]

PRN Meds Sorted by Name

fo [redacted]

Legend:

Given	Hold	Not Given	Due	Canceled Entry	Other Actions
Time	Time	(Time)	Time	Time	Time-Action

Inactive	Active	Linked
X		

Medications [redacted]

bisacodyl (DULCOLAX) suppository 10 mg Dose: 10 mg Freq: Daily as needed Route: RE PRN Reason: constipation [redacted]							
magnesium oxide (MAG-OX) tablet 400 mg Dose: 400 mg Freq: Daily as needed Route: PO PRN Comment: For serum Magnesium level between 1.8 and 2 mg/dL [redacted]							
Or magnesium sulfate in D5W IVPB (premix) 1 g Dose: 1 g Freq: Daily as needed Route: IV PRN Comment: For serum Magnesium level between 1.8 and 2 mg/dL [redacted] Admin Instructions: For patients who are NPO or cannot tolerate oral magnesium Maximum rate = 150 mg/min	X	X	X	X	X	X	X
ondansetron (ZOFRAN) injection 4 mg Dose: 4 mg Freq: Every 4 hours as needed Route: IV PRN Reasons: nausea,vomiting [redacted] Admin Instructions: If patient is NPO or unable to tolerate PO							
oxyCODONE (ROXICODONE) immediate release tablet/capsule [redacted] mg Dose: [redacted] mg Freq: Every 4 hours as needed Route: PO PRN Reason: moderate pain [redacted] [redacted]							

[redacted]

[redacted]

oxyCODONE (ROXICODONE) immediate release tablet/capsule 5 mg Dose: 5 mg Freq: Every 4 hours as needed Route: PO PRN Reason: severe pain [redacted]							
[redacted]							
sodium chloride (NS) [redacted]% flush 3 mL Dose: 3 mL Freq: Every 8 hours as needed Route: IV PRN Reason: line care							
[redacted]							
Medications [redacted]							

[redacted]

[redacted]

[redacted]

[redacted]
Physician
Surgical ICU

H&P ⚠
Addendum

[redacted]

TRAUMA ICU H&P

Name:	[redacted]	**MRN:**	[redacted]
DOB:	[redacted]	**Acct:**	[redacted]
Age:	65 y.o.	**Admit Date:**	[redacted]
Sex:	female	**Attending:**	[redacted]
PCP:	PCP, NONE, MD	**Location:**	[redacted]

Length of Stay: 0

Date of serv [redacted]

OR Case Date: * No surgery found *

Subjective / 24 hour events:
Admitted to the ICU service overnight by trauma due to automobile versus pedestrian collision resulting in significant SAH. Shew as intubated for airway protection overnight. Currently she is on minimal vent requirements. Her best GCS is [redacted](7T). She is about to go down for her repeat CTH now. She is afebrile, vss, on minimal fent and prop and HTS 3% at 20cc/hr. She was not stated on mannitol due to lactic acidemia. She was boiused overnight 1L NS for hypotension. She also has an open left tibfib fracture and is splited with dopplerable signals. Her pmh is unknown. Her EKG shows some r atrial enlargement. She has no identified family at this time. She has periorbital swelling. Her pupiles are briskly reactive b/l.

HPI

[redacted]s a 65 y.o. female homeless who was hit by a car traveling at 40 mph while she was pushing a cart and dragged with extensive damage to windshield. Person stopped to render assistance and called EMS. EMS noted AMS. C-collar applied. SBP 90s on scene, improved by time of arrival. Pt was combative and interfering with trauma workup and therefore was intubated (with succinylcholine) for airway protection. +EtOH.

Objective

Vitals:

[redacted]

BP:	(!) 143/97	115/85	(!) 117/96	[redacted]
BP Location:		Left arm		
Patient Position:		Lying		
Pulse:	80	75	79	79
Resp:	18	18		18
Temp:	[redacted] °F [redacted] °C)	(!) [redacted] °F [redacted] °C)		
TempSrc:	Oral	Oral		
SpO2:	100%	100%	100%	100%
Weight:				
Height:				
PF:				

[redacted]

[redacted]

Physical Examination
General: resting comfortably, sedated for repeat CTH. no family at bedside Psychological: encephalopathic, tried to get out of bed this am but wouldnt open eyes or follow Neurological: best GCS 7 (1-1T-5) ,alert and oriented, no focal deficits, gross sensation/motor intact HEENT: periorbital swelling b/l, trachea midline, ett and ogt in place, mucosa moist, cervical spine no step-offs/deformities and non-tender, c-collar in place Pulmonary/chest: chest rise equal b/l breathing unlabored, no audible wheezing on minimal AC vent requirements, no external trauma Cardiovascular: regular rate and rhythm, palpable radial and DP pulses b/l Gastrointestinal: abdomen soft, non-distended, no ecchymosis or external signs of trauma Back: no external signs of trauma, spine is non-tender, no step-offs/deformities Genitourinary: pelvis grossly stable and non-tender, no bloody drainage Musculoskeletal: moves all extremities spontaneously, open left tibfib fx in splint, dopplerable dp signal. Mild swelling right ankle. Skin: warm and dry, no road rash or other lacerations

DRAINS: foley, ett, ogt

I&O
No intake/output data recorded.
Net IO Since Admission: No IO data has been entered for this period [redacted]

Select Labs (last 7 days):

ABG

Results from last 7 days

Lab	Units	[redacted]
PH ART		7.350
PCO2 ART	mmHg	[redacted]
PO2 ART	mmHg	**279.7***
HCO3 ART	mmol/L	[redacted]
BASE EXC ART	mmol/L	[redacted]
O2 SAT ART	%	[redacted]
HEMOGLOBIN BG	g/dL	[redacted]
O2HB	%	[redacted]
CARBOXYHEMOGLOBIN	%	**0.3***
METHEMOGLOBIN	%	[redacted]
DEOXYHEMOGLOBIN	%	[redacted]
SAMPLE TYPE		Blood Arterial
MODE		VENT - AC
FIO2	%	50

Anemia

CBC

Results from last 7 days

Lab	Units	[redacted]

[redacted]

[redacted]

		0457
WBC AUTO	10*3/uL	[redacted]
RBC AUTO	10*6/uL	
HEMOGLOBIN	g/dL	
HEMATOCRIT	%	
MCV	fL	
MCH	pg	
MCHC	g/dL	
RDW	%	
PLATELETS AUTO	10*3/uL	216
MPV	fL	[redacted]
NEUTROPHILS RELATIVE	%	
LYMPHOCYTES RELATIVE	%	[redacted]
MONOCYTES RELATIVE	%	[redacted]
EOSINOPHILS RELATIVE	%	[redacted]
BASOPHILS RELATIVE	%	[redacted]
NEUTROS ABS	10*3/uL	[redacted]
LYMPHS ABS AUTO	10*3/uL	5.0*
MONOS ABS AUTO	10*3/uL	[redacted]
EOS ABS AUTO	10*3/uL	[redacted]
BASOS ABS AUTO	[redacted]	[redacted]

CMP

Results from last 7 days

Lab	Units	[redacted]	
SODIUM	mmol/L	--	144
POTASSIUM	mmol/L	--	[redacted]
CHLORIDE	mmol/L	--	110
CO2	mmol/L	--	[redacted]
BUN	mg/dL	--	
CREATININE	mg/dL	--	
BUN / CREAT RATIO		--	
GLOMERULAR FILTRATION RATE	mL/min/1.73m*2	--	75.1*
AST	U/L	--	91*
ALT	U/L	--	34*
ALK PHOS	U/L	--	78
ALBUMIN	g/dL	--	[redacted]
BILIRUBIN TOTAL	mg/dL	--	
CALCIUM	mg/dL	--	[redacted]
LACTATE	mmol/L	3.85*	

Results from last 7 days

Lab	Units	[redacted]

[redacted]

[redacted]

		0457
GLUCOSE	mg/dL	143*

Cardiac

Results from last 7 days

Lab	Units	[redacted]
HS TROPONIN I	ng/L	6

Coagulation

Results from last 7 days

Lab	Units	[redacted]
PROTIME	seconds	[redacted]
INR		0.99
PTT	seconds	22.2

Thyroid Function / HGBA1C

Urine

Micro

No results found for: "AERCULTLC", "AFBCULTURE", "ANACX", "BLOODCX", "BONECX", "BRONCHIALCX", "CAMPY1", "CATHETERCX", "CAURISCX", "CRESCREENCX1", "CSFCX", "CULTURE", "CXBPERTUSSIS", "EARCULTURE", "EYECULTURE", "FUNGCX", "GCCX", "GENITALCULTU", "LABENVI", "LABFUNG", "LABGCC", "MRSACULTURE", "MYCOPLASCX"No results found for: "NASALCX", "NOSECULTURE", "RECT1", "SPUTUMCULTUR", "STOOLCX", "STREPACULT", "STREPACX", "STREPBCULT", "THROATCX", "TISSUECULTUR", "TRANREXCX1", "TRANSRXCX1", "UPPERGICX", "URINECX", "VRECX", "WOUNDCX", "YERSINIACULT"

Imaging (last 48 hours)

X-ray Chest 1 View
Result [redacted]
ET tube tip is below the thoracic inlet and above the carina. No infiltrates are visualized. No pleural effusion or pneumothorax. Cardiac silhouette is normal in size. The pulmonary vascularity is within normal limits.

X-ray Pelvis 1 View
Result [redacted]
FINDINGS/IMPRESSION: No radiographic evidence of acute fracture or dislocation is seen. No significant degenerative changes. No gross soft tissue swelling.

X-ray Tibia Fibula Left
Result [redacted]
FINDINGS/IMPRESSION: Acute, comminuted, displaced fracture of the distal tibial diaphysis. Acute, comminuted, displaced fracture of the distal fibular diaphysis. Soft tissue swelling and gas in the anterior overlying soft tissues is present.

CT Angiogram Left Lower Extremity with and/or without Contrast
Result [redacted]
: Acute fracture distal tibia, distal fibula as described above Vasculature normal subcutaneous emphysema This preliminary report wa[redacted]

CT Head without Contrast
Result [redacted]

[redacted]

[redacted]

: Acute subarachnoid ,subdural ,intraparenchymal hemorrhage as described above. This preliminary report was electronically signed [redacted]

CT Cervical Spine without Contrast
Result [redacted]

CT Chest Abdomen Pelvis W IV Contrast Only
Result [redacted]
: Negative chest Rectal wall thickening, differential includes proctitis, distention of the rectosigmoid, stercoral colitis cannot be excluded This preliminary report was [redacted]

Current Medications:

Scheduled Meds

• bacitracin		Topical	TID
• ceFAZolin (ANCEF) IV	2 g	Intravenous	Q8H
• dextrose 5 % with ceFAZolin (ANCEF) - ADS Override Pull			
• famotidine	20 mg	Intravenous	BID
• folic acid	1 mg	Oral	Daily
• levETIRAcetam (KEPPRA) IVPB	500 mg	Intravenous	Q12H
• multivitamin	1 tablet	Oral	Daily
• potassium chloride IVPB	40 mEq	Intravenous	Once
• thiamine	100 mg	Intravenous	Daily
• [START ON [redacted]	100 mg	Oral	Daily

Infusing Meds

• fentaNYL	[redacted] mcg/hr	[redacted]
• Propofol	[redacted] mcg/kg/min	
• sodium chloride	20 mL/hr	

PRN Meds

dextrose 5 % with ceFAZolin (ANCEF) - ADS Override Pull, ,
hydrALAZINE, 10 mg, Q2H PRN
labetalol, 5 mg, Q2H PRN
magnesium oxide, 400 mg, Daily PRN
Or
magnesium sulfate IVPB, 1 g, Daily PRN
magnesium sulfate IVPB, 2 g, Daily PRN
magnesium sulfate IVPB, 4 g, Daily PRN
ondansetron, 4 mg, Q4H PRN
potassium & sodium phosphates, 1 packet, BID PRN
Potassium Bicarb, 20 mEq, Daily PRN
Potassium Bicarb, 40 mEq, Daily PRN
Potassium Bicarb, 60 mEq, Daily PRN

[redacted]

[redacted]

potassium chloride IVPB, 20 mEq, Daily PRN
Or
potassium chloride IVPB, 40 mEq, Daily PRN
Or
potassium chloride IVPB, 60 mEq, Daily PRN
sodium chloride, 3 mL, Q8H PRN
sodium phosphate IVPB, 15 mMol, Daily PRN
Or
sodium phosphate IVPB, 30 mMol, Daily PRN

Assessment

ICU LOS:
1h

65 y.o. female s/p auto versus pedestrian

INJURY DA[redacted]

MECHANISM: automobile versus pedestrian collision

CONSULTS:
Neurosurger[redacted]
Orthopedic surge[redacted]

OPERATIONS:
[redacted]Pending ventriculostomy and craniectomy by neurosurgery

PROCEDURES:
[redacted]ETT (ED)
Closed reduction with manipulation of L tib/fib fx, short posterior splint application.
Occipital scalp stellate lac simple repair (5 cm)

LIST OF INJURIES:
Blunt head trauma
SAH bil frontoparietal as well as suprasellar, ambient, pre-pontine, and retrocrural cisterns
SDH interhemispheric fissure and bilateral tentorium
IPH cerebral peduncle
Midline shift R to L (3.1 mm)
Closed R occipital bone comminuted fx
Occipital cephalohematoma
Occipital scalp lac
Blunt thoracoabdominal trauma
Open L distal tib fib comminuted and displaced fx
Abrasions to bil buttocks, sacrum, R knee

OTHER DIAGNOSES:
Acute pain due to trauma
Right atrial enlargement
Acute respiratory failure requiring intubation
Acute dysphagia requiring OGT
Open left fx of tibia and fibula
Lactic acidemia
Hypokalemia

PMH / HOME MEDS:

[redacted]

[redacted]

unknown

Plan

SYSTEMS-BASED ASSESSMENT/PLAN:

NEURO:
Acute pain from trauma
Continue fentanyl gtt and titrate to nonverbal pain score / cpot <=2
Blunt head trauma
SAH bil frontoparietal as well as suprasellar, ambient, pre-pontine, and retrocrural cisterns
SDH interhemispheric fissure and bilateral tentorium
Acute metabolic encephalopathy
Midline shift R to L [redacted] mm)
Closed R occipital bone comminuted fx
Occipital cephalohematoma
Occipital scalp lac
Neurosurgery consulted. Q1hr neurochecks. rCTH q4-6hrs until stable. No AC for reversal. Elevate HOB >30deg. 1g keppra given. Continue 500mg bid a 7days
Avoid hypotension, target SBP [redacted] glucose <200, normothermia, pc02 [redacted] p02 >80.
No signs of cerebral edema but there is slight midline shift. Target sodium [redacted]
3% HTS at 20cc/hr. Mannitol held due to hypovolemia overnight. Trend Ns and serum osmol q6hrs
Repeat CTH this am shows slight worsening of SAH and the patient has a stably reduced GCS.
Neurosurgery first planning for ventriculostomy and now planning for craniectomy
F/u postop evaluation and rCTH.

HEENT:
CT Cspine negative for acute fxs. Ccollar removed

CV:
Blunt thoracoabdominal trauma
Right atrial enlargement
Borderline hypotensive this am
Baseline EKG shows r atrial enlargement. Unknown if cardiac hx
Obtain echo. If abnormal consult cardiology
Monitor vs on cardiac telemetry

PULM:
Acute respiratory failure requiring intubation
No chest trauma on CT
Bronchopulmonary hygiene. Encourage deep breathing

GI:
Acute dysphagia requiring OGT
NPO until no procedures then start tube feeds
Bowel regimen with colace, senna. Prn suppository if no bm in 3 days

GU/FEN:
Lactic acidemia
Hypokalemia
Trend LA q6hrs until [redacted]
K already replaced
On HTS 20cc/hr. Q6hr cmp keep na [redacted]
Strict i/o q 4hrs. Goal uop 0.5cc/kg/hr. Trend daily BUN/Cr
Trend lytes on daily CMP. Replace prn to keep [redacted]
Foley for critical monitoring

HEME:
No anemia or coagulopathy from baseline labs

[redacted]

[redacted]

No transfusions to date. Trend h/h on daily CBC. Type and screen done

ID:
Open blunt fracture
Continue ancef 2g q8hrs
Trend wbc on daily cbc. If [redacted] >101.5F send blood cxs

ENDO:
No known hx of DM or chronic steroids. Goal glucose <200.
Obtain hgba1c, tsh, lipids, bnp
If glucose >200 send HgbA1c / start POCT/SSI q6hrs or ac/hs

MSK:
Open L distal tib fib comminuted and displaced fx
Abrasions to bil buttocks, sacrum, R knee
Orthopedic surgery consulted. Awaiting neurosurgery clearance
Would also prefer to get echo first since no hx and [redacted] have rhf per ekg
Repeat EKG now as well to r/o prolonged QT
Regular skin checks. Offload q2hr to prevent DTI
PT/OT consults postop

PROPHYLAXIS:
VTE:
Lovenox 30mg q12 hrs - dont start for at least 24 hrs post stable cth
SCD to b/l LE while in bed
GI:
Pepcid 20mg IV bid for stress ulcer ppx

TUBES/LINES/DRAINS:
PIV x 2

DISPO:
Admit to trauma ICU under trauma service

Goals for next 24 hours:
Ventric placement
Postop repeat CTH
Trend serum Na and osmol and lactic acid q6hrs
If no surgery later today or tomorrow start tube feeds per dietician recs\
OR with orthopedic surgery once cleared from neurosurgical standpoint

Critical Care Attestation:
Total Critical Care time: 75 minutes
Due to a high probability of clinically significant, life-threatening deterioration, the patient required my highest level of preparedness to intervene in an emergent fashion and I personally spent this critical care time directly and personally managing the patient. This critical care time included obtaining a history; examining the patient; pulse oximetry; ordering and review of studies; arranging urgent treatment with development of a management plan; evaluation of patient's response to treatment; frequent reassessment; and performing discussions with other consultants/providers. This critical care time was performed to assess and manage the high probability of imminent, life-threatening deterioration that could result in multi-organ failure. It was exclusive of [redacted] billable procedures and treating other patients and teaching time. Please see H/P, labs, imaging sections for further information on patient assessment and treatment.

DATE OF SERVICE [redacted]

[redacted]

Trauma, Acute Care Surgery, Critical Care

Regarding surgical and trauma ICU inpatients:
Non-emergent issues during the day: SICU NP [redacted]

[redacted]

[redacted]

Emergent issues anytime: ACS/SICU MD [redacted]

Regarding surgical and trauma FLOOR inpatients:
Non-emergent issues during the day: Trauma NP [redacted]
Emergent issues anytime: Trauma MD ([redacted]

Regarding NEW surgical and trauma patients:
New surgical consults/immediate postops: ACS/SICU MD [redacted]
New trauma consults/activations: Trauma MD [redacted]

Non-emergent trauma pt questions (anywhere): Trauma RN Clinician [redacted]

Revision History

Date/Time	User	Provider Type	Action
[redacted]		Physician	Addend
		Physician	Sign

[redacted]

[redacted]

[redacted]

[redacted]
Nurse Practitioner
Trauma

Progress Notes
Cosign Needed

[redacted]

TRAUMA PROGRESS NOTE

Name:	[redacted]	**MRN:**	[redacted]
DOB:	[redacted]	**Acct:**	[redacted]
Age:	55 y.o.	**Admit Date:**	[redacted]
Sex:	female	**Attending:**	[redacted]
PCP:	PCP, NONE, MD	**Location:**	[redacted]

Date/ Surgeon [redacted]

Subjective

-Alert and oriented x 3, following commands, no complaints
Na level WNL
-able to transfer onto commode and bed with 2 person assist - SNF placement
-DC order placed [redacted] lt SNF placement- Recup acre order placed again
-Spoke with PT [redacted] atient for wheelchair transfers, last see[redacted]
-New PT order place[redacted]

Objective

Vitals

	Current	24 Hour Min / Max
Temp	98 °F [redacted] °C)	Temp Min: 98 °F [redacted] °C) Max: [redacted] °F [redacted] °C)
BP	(l) [redacted]	BP Min: [redacted] Max: 109/70
Pulse	(l) 101	Pulse Min: 80 Max: 103
Resp	17	Resp Min: 17 Max: 19
SpO2	97 %	SpO2 Min: 0 % Max: 99 %
Weight	48 kg (105 lb [redacted] oz)	Initial Weight 48 kg (105 lb [redacted] oz)
	Body mass index is [redacted] kg/m².	

I&O

I/O last 3 completed shifts:
In: 956 [P.O.:956]
Out: 600 [Urine:600]

Net IO Since Admission: 12,886.19 mL [redacted]

Physical Examination	
General:	No acute distress
Neuro:	MAE. Follows commands
Head:	R scalp incision w/ staples C/D/I
Eyes:	Pupils equal, round, reactive to light Extraocular muscles Intact Periorbital ecchymosis, left eye
ENT:	Ears normal in appearance, atraumatic Bilateral nares clear, no blood noted Soft Supple Trachea in midline
Chest wall:	No chest wall deformity

[redacted]

[redacted]

	Chest non-tender to palpation
Pulmonary:	Lungs clear to auscultation bilaterally
Abdomen:	Soft, flat, non-distended, non-tympanic
Extremities:	Upper Extremities: Atraumatic Lower Extremities: LLE with surgical wounds with staples, c/d/, mild swelling, some bruising.
Vascular:	DTS--Vascular Pulses Exam: Normal

Select Labs (last 7 days):

ABG

Anemia

CBC

Results from last 7 days

Lab	Units	[redacted]		
WBC AUTO	10*3/uL			4.30
RBC AUTO	10*6/uL			
HEMOGLOBIN	g/dL			
HEMATOCRIT	%	30.9*	29.1*	
MCV	fL			
MCH	pg			
MCHC	g/dL			
RDW	%			
PLATELETS AUTO	10*3/uL	427	462*	509*
MPV	fL	6.5*	6.0*	6.0*

CMP

Results from last 7 days

Lab	Units	[redacted]	
SODIUM	mmol/L	140	143
POTASSIUM	mmol/L		
CHLORIDE	mmol/L	101	105
CO2	mmol/L		
BUN	mg/dL		
CREATININE	mg/dL		
BUN / CREAT RATIO			
GLOMERULAR FILTRATION RATE	mL/min/1.73m*2		>90.0
AST	U/L	24	39*
ALT	U/L	47*	60*
ALK PHOS	U/L	142*	133*
ALBUMIN	g/dL		
PHOSPHORUS	mg/dL		
MAGNESIUM	mg/dL		
BILIRUBIN TOTAL	mg/dL		
CALCIUM	mg/dL		

[redacted]

[redacted]

Results from last 7 days

Lab	Units	[redacted]				
GLUCOSE	mg/dL	94	87	88	87	108

Cardiac

Coagulation

Thyroid Function / HGBA1C

Urine

Micro

MRSA CULTURE

Date	Value	Ref Range	Status
[redacted]	No Methicillin Resistant Staphylococcus aureus Isolated		Final

No results found for: "NASALCX", "NOSECULTURE", "RECT1", "SPUTUMCULTUR", "STOOLCX", "STREPACULT", "STREPACX", "STREPBCULT", "THROATCX", "TISSUECULTUR", "TRANREXCX1", "TRANSRXCX1", "UPPERGICX", "URINECX", "VRECX", "WOUNDCX", "YERSINIACULT"

Imaging (last 48 hours)

No results found.

Current Medications:

Scheduled Meds

• acetaminophen	1,000 mg	Oral	TID
• bacitracin		Topical	TID
• enoxaparin (LOVENOX) injection	30 mg	Subcutaneous	Q12H
• levETIRAcetam	500 mg	Oral	Q12H SCH
• melatonin	3 mg	Oral	HS
• methocarbamol	500 mg	Oral	TID
• multivitamin	1 tablet	Oral	Daily
• QUEtiapine	100 mg	Oral	BID
• senna-docusate	1 tablet	Oral	BID
• sodium chloride	3 g	Oral	TID
• urea	15 g	Oral	TID

Infusing Meds

[redacted]

[redacted]

PRN Meds

bisacodyl, 10 mg, Daily PRN
magnesium oxide, 400 mg, Daily PRN
ondansetron, 4 mg, Q4H PRN
oxyCODONE, [redacted] Q4H PRN
oxyCODONE, 5 mg, Q4H PRN
sodium chloride, 3 mL, Q8H PRN

Assessment

55 y.o. female s/p AVP

INJURY DATE[redacted]
MECHANISM: AVP

INJURIES:
Blunt head trauma
SAH bil frontoparietal as well as suprasellar, ambient, pre-pontine, and retrocrural cisterns
SDH interhemispheric fissure and bilateral tentorium
IPH cerebral peduncle
Midline shift R to L [redacted] mm)
Closed R occipital bone comminuted fx
Occipital cephalohematoma
Occipital scalp lac
Blunt thoracoabdominal trauma
Open L distal tib fib comminuted and displaced fx
Abrasions to bil buttocks, sacrum, R knee
Hyponatremia

OTHER DIAGNOSES:
Acute pain due to trauma
Right atrial enlargement
Acute respiratory failure requiring intubation
Acute dysphagia requiring OGT
Open left fx of tibia and fibula
Lactic acidemia
Hypokalemia

CONSULTS:
NSGY
ORTHO

OPERATIONS:
[redacted]Right Ventriculostomy placement

[redacted]Left distal tibia fracture nailing
Open reduction internal fixation of the left intra-articular pilon fracture

PROCEDURES:
[redacted]ETT (ED)
Closed reduction with manipulation of L tib/fib fx, short posterior splint application.
Occipital scalp stellate lac simple repair (5 cm)

Plan

[redacted]

[redacted]

#Acute pain from trauma
-Tylenol 1 gm q8 hr PO
-Oxy 2.5mg/5 PO PRN
-Robaxin 500mg PO TID

#Blunt head trauma
#SAH bil frontoparietal as well as suprasellar, ambient, pre-pontine, and retrocrural cisterns
#SDH interhemispheric fissure and bilateral tentorium
#Acute metabolic encephalopathy
#Midline shift R to L [redacted] mm)
#Closed R occipital bone comminuted fx
#Occipital cephalohematoma
#Occipital scalp lac
-Neurosurgery consulted.
-Q4hr neurochecks, rCTH q4-6hrs until stable. No AC for reversal.
-Elevate HOB >30deg.
- 1g keppra given on admission. Then 500mg bid - completed
-Continue Salt tabs, last Na 139
-s/p right ventriculostomy [redacted]
[redacted] Repeat CT head stable

Agitation
Seroquel 100 mg BID

#Blunt thoracoabdominal trauma
#Right atrial enlargement
-HDS
-ECHO: EF [redacted]

-No chest trauma on CT
-Extubated [redacted]
-on RA, no issues
-IS, pulm hygiene

#Acute dysphagia
-dietary following, on dietary supplements
-Currently on puree diet
-Bowel regimen with colace, senna.

No acute issues
Monitor UOP

#Open L distal tib fib comminuted and displaced fx
#Abrasions to bil buttocks, sacrum, R knee
-Orthopedic surgery consulted
-s/p IMN tibi [redacted]
-NWB LLE
-ASA 81mg BID for DVT ppx x 4 weeks after discharge
- Okay to leave incisions open to air after removal.
-follow up with [redacted] weeks in ortho trauma clinic.
-PT/OT
-SW

#R ankle bruising, swelling
#R medial malleolus fracture
-ortho consulted
-Non-op
-WBAT RLE in walker boot

#Hyponatremia- resolved

[redacted]

[redacted]

#SIADH
-Total clear fluid restriction of 1,000ml/24 hr
-Strict i/o q4hr (po intake, ivf, uop)
-Daily weights
-TSH, T4 WNL
-Serum osmo low, urine osmo high normal, urine sodium high normal - consistent w/ SIADH
-continue Na tabs 3 gm TID
-Avoid diuretics and hypotonic IVF

Diet: Puree/ mildly thick liquids - 1000 ml fluid restriction
Fluids: None

Intake/Output Summary (Last 24 hours[redacted]
Last data filed at[redacted]

	Gross per 24 hour
Intake	720 ml
Output	600 ml
Net	**120 ml**

HEM/ID
Antibiotics: None

Lines: PIV
Foley: None

VTE prophylaxis: Lovenox 30mg BID
GI Prophylaxis: pepcid 20mg daily

Social work: Trauma assessment
Case manager: recup care/ patient is homeless, SNF order also placed[redacted]since[redacted]till requires 2 person assist
Dispo: SNF- DC placed[redacted]medically cleared

[redacted]SW attempted to f/u with[redacted]ovide clarification regarding
n and inform that prior outreach efforts, including welfare check, were unable to confirm
ation. MSW was informed information can not be released due to HIPAA restrictions. MSW
ed with supervisor and was advised to email[redacted]social media post. Email sent. SS to remain
le
egarding SS consult for "find NOK", MSW conducted chart review an[redacted]as of[redacted]pt's
er was identified and at bedside; contact info obtained and now reflecting on pt's facesheet. MSW
[redacted]who confirmed[redacted]s been identified and contact info is available. No further
erved to be needed at this time. CONSULT COMPLETED AND CLEARED.
M order placed for WC w/ ELR, SW consult placed for shelter placement for DC
scussed with CM, needs manual wheel chair with removable legs. Plan for discharge to Recoop for

heel chair at bedside.
o follow up on re[redacted]re
[redacted]ent clinicals
CP acknowledge[redacted]r for snf palcement. Orders have been sent out. See destination tab
[redacted]was refer for more snf no acceptance barrier homeless

Author:
[redacted]

[redacted]

[redacted]

[redacted]

[redacted]
Occupational Therapist
Specialty: Occupational Therapy

Occupational Therapy
Signed

Date of Service [redacted]

Occupational Therapy Treatment Note

Patient Name [redacted]

Pt seen on this d[redacted] for OT treatment. Chart reviewed[redacted] cleared patient for OT. Patient agreed to participate with therapy.

OT Recommendation

Recommendation
Recommendation: Post-acute Care (Pt will benefit from daily therapy.)
Equipment Recommended: Defer to next LOC.
Additional Comments
Additional Comments: SW consult as pt is homeless.

Problem List
Patient Active Problem List
Diagnosis
- SAH (subarachnoid hemorrhage) (CMS/HCC)
- Tibia/fibula fracture
- Hyponatremia

Precautions: Fall risk, impulsive, HOB >30 deg, target SBP [redacted] NWB on LLE (s/p L distal tib-fib nailing ORIF), WBAT RLE (R medial malleolus fx - non op) in CAM boot, s/p fall in hou[redacted] dizziness, orthostatic hypotension (monitor vitals)

Subective

Pain
Pain Assessment
Pain Assessment: No/denies pain
Pain Score: 0

Vitals
Vital Signs
Heart Rate: 95
Heart Rate Source: Blood Pressure Machine
SpO2: 98 %
BP: 116/72
MAP (mmHg): 81
BP Location: Right arm
BP Method: Automatic
Patient Position: Lying

Objective

ROM/MMT
Right Upper Extremity Assessment

[redacted]

[redacted]

RUE Assessment
RUE Assessment: Within Functional Limits (AROM WFL; MMT: 3+/5 grossly)

Left Upper Extremity Assessment
LUE Assessment
LUE Assessment: Within Functional Limits (AROM WFL; MMT: 3+/5 grossly)

Balance
Balance
Sitting Static: good
Sitting Dynamic: good (-)
Standing Static: fair (+)
Standing Dynamic: fair
Comments: in standing with FWW

Activity Tolerance
Activity Tolerance
Endurance: Tolerates 30 min activity with multiple rests
Activity Tolerance Comments: limited by generalized weakness and reported fatigue

Coordination
Coordination
Coordination: WFL

Sensation
Sensation
Light Touch: No apparent deficits
Numbness: No
Numbness - Comments: B feet
Paresthesias: No
Paresthesias - Comments: B feet

Cognition/Communication
Cognition
Orientation Level: Oriented to place, Oriented to person, Disoriented to time, Disoriented to situation
Deficits: [redacted] awareness of deficits
Comments: pt followed ~80% of simple one-step commands

Activities of Daily Living
Current ADL
Eating Assistance: Supervision
Grooming Assistance: Moderate assist
UE Dressing Assistance: Minimal assist
LE Dressing Assistance: Maximum assist, Minimal assist
Toileting Assistance with Device: Contact guard

Current Functional Mobility
Bed Mobility
Rolling: Stand by assist
Supine to Sit: Minimal assist
Sit to Supine: Stand by assist
Assistive Device: Bed rail, 1 person
Mobility: Stand by assist

Current Functional Transfers
Transfers
Sit to Stand: Minimal assist
Toilet Transfer: Minimal assist to right, Minimal assist to left (stand pivot to BSC)
Assistive Device: 1 person, Gait belt, Front wheel walker

[redacted]

[redacted]

Assessment

Pt seen for OT tx session, cleared by [redacted] Pt received asleep in bed, semi-fowler position. Pt reported fatigue due to lack of sleep, initially refusing treatment, however agreeable following education of benefits of early mobility. Pt A&Ox2, oriented to person and place, denies numbness/tingling and dizziness, and reports [redacted] pain. Pt followed ~80% of simple one-step commands with intermittent need for redirection to task. Pt donned gown in supine with minA to bring sleeves to shoulders d/t generalized weakness. Pt reported unaware of NWB LLE precautions thus re-educated with immediate teach-back. Pt transitioned from supine to EOB with HHA and head of bed elevated. Pt able to don CAM boot to RLE EOB with maxA and no noted LOB with no use of BUE on mattress. Pt completed STS transfer with FWW with minA and able to perform stand pivot transfer into wheelchair. Pt refused self-propelling in wheelchair thus engaged pt in LB dressing with minA to don over B toes with pt able to pull over heels. Pt able to perform w/c <> bed transfer via stand pivot with minA with max verbal cues to adhere with WB precautions however with fair carryover. Pt returned to bed with SBA. During tx session, rest breaks taken as needed. Pt tolerated tx session well. No adverse events noted during tx session.

Pt educated on: compensatory techniques, pain management, energy conservation techniques, benefits/role of skilled OT, safety during ADLs / functional transfers / functional mobility, safe use of DME, benefits of AE/AD/DMEs, proper hand/foot placements, proper body mechanics.
Pt verbalized understanding of education.
Pt will benefit from further eduction.

Pt left in bed, positioned for comfort / pressure relief, bed alarm on, call light and all needs within reach, emphasized not to get out of bed without nursing assist. Nurse informed of transfer of care, nurse agreed to resume care.

OT Goals
OT Goals
ADL's - Pt. will be able to perform meal task:: Modified independent
ADL's - Pt. will be able to perform grooming:: Modified independent
ADL's - Pt. will be able to perform UB dressing:: Modified independent
ADL's - Pt. will be able to perform LB dressing:: Modified independent
ADL's - Pt. will be able to perform toileting:: Stand by assist
Bed Mobility/Functional Transfers - Pt. will be able to perform bed rolling:: Modified independent
Bed Mobility/Functional Transfers - Pt. will be able to perform supine to sit:: Modified independent
Bed Mobility/Functional Transfers - Pt. will be able to perform sit to stand:: Contact guard assist
Bed Mobility/Functional Transfers - Pt. will be able to perform toilet transfer:: Contact guard assist

Plan

OT Recommendation and Plan
Plan
Treatment/Interventions: ADL retraining, Functional transfer training, UE strengthening/ROM, Patient/family training, Neuromuscular reeducation, Fine/gross motor, Compensatory technique education, Continued evaluation, Therapeutic exercise, Therapeutic activity
OT Frequency: 3-5x/wk
Duration: 2 weeks
Recommendation
Recommendation: Post-acute Care (Pt will benefit from daily therapy.)
Equipment Recommended: Defer to next LOC.

Plan to continue OT POC.

Gait belt applied during session: Yes

[redacted]

[redacted]

[redacted]

[redacted]

[redacted]
Physical Therapy Assistant

Physical Therapy Signed

Date of Servic[redacted]

Patient is seen for continuing therapy treatment. Patien[redacted], RN and agreed to participate with therapy.

PHYSICAL THERAPY DAILY TREATMENT NOTE

Today's Date[redacted]

Patient Name[redacted]
Patient's age: 55 y.o.
Room[redacted]

Precautions
Precautions
Weight Bearing Status: WBAT R LE in CAM Boot + NWB LLE
Fall Precautions: Yes (fell o[redacted]
Assistive Device Needed: manual WC + FWW
Additional Comments: confused; impulsive; poor safety awareness; S/P ORIF L LE-NWB (non compliant), WBAT R LE in CAM Boot[redacted] Hyponatremia, AVP, bed mob: SUP/SBA, (previously STS min A w/ FWW, GT mod A 2 small steps fwd/back w/ FWW)

Summary:
Pt received in semirecumbent position. Pt endorses[redacted] pain upon encounter, and denies dizziness or N/T symptoms. Nursing endorsed about pt complaint of pain. Vitals assessed in supine and were WFL. Pt asked if she remembers weight bearing precautions, which pt denied, requiring re-education on LLE NWB and WBAT RLE in CAM boot precautions. Pt verbalized understanding. Today, pt participated in bed mobility and BLE TE in supine. Once seated at EOB, pt endorsed feeling dizzy. Pt participated in seated BLE TE to facilitate blood circulation. Pt continued to endorse dizziness. Vitals taken and noted BP drop from 118/79 in supine to[redacted] Deferred OOBA d/t orthohypo symptoms for safety. Assisted pt back to bed. Vitals re-assessed and BP 109/77. All safety measures in place post therapy. Endorsed.

SUBJECTIVE

Pain
Pain Assessment
Pain Assessment:[redacted]
Pain Score: 6

Vital Signs
Vital Signs
Heart Rate: 95
Heart Rate Source: Blood Pressure Machine
Resp: 17
SpO2: 98 %
BP: 118/79[redacted] when seated)
MAP (mmHg): 89
BP Location: Left arm
BP Method: Automatic
Patient Position: Lying

Language
Communication
Primary Language Spoken: English
Oral Communication: Intact

[redacted]

[redacted]

CURRENT FUNCTIONAL MOBILITY ASSESSMENT

Bed Mobility
Rolling: Supervision
Supine to Sit: Stand by assist
Sit to Supine: Stand by assist
Assistive Device: Bed rail, 1 person
Mobility: Stand by assist

Transfers
Sit to Stand: Unable to assess (Deferred)

Gait
Ambulation: Unable to assess (Deferred)

Balance
Sitting Static: fair (+)
Sitting Dynamic: fair
Standing Static: other (comments)
Standing Dynamic: other (comments)
Comments: Assessed seated at EOB only

Additional Neuromuscular Education
Facilitation for:: Posture/dynamic trunk
Posture/dynamic trunk: symmetrical weight bearing, midline orientation, weight shifting, tactile cues, verbal cues

Exercises
Exercises: TE, TA, NMR

PLAN

TREATMENT PLAN OF CARE:
Plan
Treatment/Interventions: Functional transfer training, LE strengthening/ROM, Endurance training, Bed mobility training, Gait training, Therapeutic exercises, Neuromuscular re-education
Progress: Slow progress, medical status limitations
PT Frequency: 3-5x/wk
Duration: 2 weeks
PT Plan and goals discussed and agreed upon with patient and/or family: Yes
Priority Level: Medium

TREATMENT GOALS:
PT Goals
Pt. will be able to roll:: Stand by assist
Pt. will be able to perform supine to sit:: Stand by assist
Pt. will be able to transfer sit to stand:: Contact guard assist
Pt. will be able to transfer bed to chair/wheelchair:: Min assist
Functional Mobility Goals - Pt. will be able to ambulate:: Min assist
Pt. will be able to ambulate - Device: FWW
Pt. will be able to ambulate - Distance: 25
Strength Goals:: Pt to improve Strength in B LE by at least [redacted] a grade over next 2 weeks
Balance Goals:: Improve Static & dynamic sitting & Standing balance from mod A to min A in next 2 wks
Precautions/Education:: Falls prevention education. Special edu for turning to sit, feel the back of legs to bed/chair, reach with one hand on SPC/FWW & one hand for bed/chair.
Patient Care Giver Education: daughter over phone
Misc. Goals:: PT Goal 1
PT Goal 1:: Pt will complete WC mobility/self propulsion 100ft independently in 2 weeks

[redacted]

[redacted]

D/C RECOMMENDATIONS:
Patient [redacted] benefit from the following
Recommendation
Recommendation: Post-acute Care (pt will benefit from daily therapy; SW consult for safe disposition, pt is homeless; pt unable to ambulate)
Equipment Recommended: Manual WC w/Leg rest + FWW

Patient completed therapeutic interventions under the supervision of PTA. Continue current PT POC.

Patient left comfortable in bed with all needs met.
Cosigned by [redacted]

Revision History

Date/Time	User	Provider Type	Action
[redacted]		Physical Therapist	Cosign
		Physical Therapy Assistant	Sign

[redacted]

[redacted]

@ 1:01 PM

ADMISSION/REGISTRATION

[redacted]

Admit/Serv Dt Time	IP Admit Dt/Time	ED Arr Dt Time	Disch Dt Time	Room/Bed	Location	Service
[redacted]		[redacted]		[redacted]		General Medicine

Pt Class	Arrival Mode	Source	Priority	Primary Care Physician	Office Phone
Inpatient	ALS Ambulanc	Trauma	Emergency	None Pcp, MD	None

Chief Complaint	Admission Diagnosis	User
	Tibia/fibula fracture [S*	[redacted]

Emergency Physician	Office Phone	Attending Physician	Office Phone	Admitting Physician	Office Phone

[redacted]

PATIENT

SSN [redacted]

[redacted]	Sex	MS	Religion	VIP	Conf
55 y.o.	F	Unknown	Unknown		

Race	Ethnicity	Preferred Language	Maiden Other Name
Other	Not Hispanic	English	[redacted]

[redacted]

Alternative Address:
Care of:

Home Phone: [redacted]

PATIENT EMPLOYER

Employer:
Address:

Work Phone:
Occupation:

GUARANTOR

Name: [redacted]
Address: [redacted]

Home Phone:
Relationship to Patient: [redacted]

GUARANTOR EMPLOYER

Address:

Work Phone: [redacted]

EMERGENCY CONTACT 1

Relationship to Patient: Daughter

Name: [redacted]
Address:
Home Ph:
Work Ph:
Cell Ph: [redacted]

EMERGENCY CONTACT 2

Relationship to Patient: Daughter

[redacted]
Address:
Home Ph:
Work Ph:
Mobile ph: [redacted]

Insurance # 1

Payor/Plan: MISC AUTO INSURANCE/MISC AUTO INSURANCE

Address: [redacted]

Subscribe [redacted]
Pat Rel to Subscriber: Self
Group Name: [redacted]
Group Numb [redacted]
Subscriber I [redacted]

Authorization

Auth:

[redacted]

ILLUMINATION Recuperative Care Referral Form

HEALTH + HOME

To refer a patient, please complete and submit this form with supporting documents via email to [redacted] or FAX to [redacted] For referral questions, please contact an Illumination Referral Coordinator at [redacted]

Referring Party

Referring Hospital: [redacted] Dept/Floor: Social Services.
SW/CM/RN: [redacted] Phone/Pager #: [redacted]
Email [redacted] Nursing Station #:
Authorized by: Phone #:

Patient Information

Patient Name [redacted]
Gender: ☐ Male ☒ Female ☐ Transgender ☐ Other Ethnicity: Not Hispanic
Last 4 # of Social Security [redacted] Insurance [redacted] Preferred Language: English
Housing Situation: ☒ Currently Unhoused ☐ 4+ episodes of homelessness in the past 3 years

Hospital Admission Details

Chief Complaint Admitting Diagnosis:
trauma auto vs. pedestrian
Admit Date [redacted] Any Wounds? ☐ Yes ☒ No
If yes, explain the number, location, size and stage:

Independent with Wound Care? ☐ Yes ☐ No If no, will Home Health be ordered? ☐ Yes ☐ No
Was PPD/TB Test or Chest X-Ray Performed? ☐ Yes ☐ No If yes, Date: Results:

History of Health & Living Situation

Does the patient require or have... (Please circle Y or N and provide explanation accordingly.)

O2	☐ Yes* ☒ No	Active Infection	☐ Yes* ☒ No
*Explain		*Explain	
Self Administer All Meds	☐ Yes ☐ No*	Ambulatory	☐ Yes ☒ No
Continent of Bowel & Bladder	☐ Yes ☐ No	Assistive Device	☒ Yes* ☐ No
Colostomy/Ileostomy	☐ Yes ☒ No	*Explain WC | FWW, PT eva [redacted]	
Foley Catheter	☐ Yes ☒ No	Hospice	☐ Yes* ☐ No
Diabetes	☐ Yes ☒ No	*Explain	
Insulin	☐ Yes ☒ No	Car	☐ Yes ☒ No
Diabetes related Oral Meds	☐ Yes ☒ No	Service Animal	☐ Yes ☒ No

Behavioral and Mental Health Status

Does the patient have any limitations, behavioral or mental health challenges? ☒ No ☐ Yes

☐ Mental Health DX If checked, what type?
☐ Non-compliant ☐ Forgetful
☐ Cognitive Impairment ☐ Registered Sex Offender
☒ Other Explain anxious

Length of Stay Authorized in Recupertive Care: Days

ILLUMINATION STAFF ONLY

Date Received: Reviewed by:
Approved: ☐ Yes* ☐ No** *Date: *Authorization #: **Reason for Denial:
Admission Date: Admission Time: Exit Date:
Extension Request: ☐ Yes ☐ No If YES, Additional Approval Date: by Total LOS